



PREHOSPITAL VENTILATOR PROGRAM · THREE YEARS IN

Breathing room for a rural county.

In 2023, Greene County Paramedics put transport ventilators in three fly cars to prove an idea. By 2025, every car carried one. Here is what it has meant for the people we serve.

BY STEVE NEAR, CHIEF OF OPERATIONS · OCTOBER 2026

THREE YEARS IN

147

patients ventilated in the field since June 2023.

Three ventilators in 2023 to prove the idea. A Hamilton T1 in every fly car by 2025.
And zero breathing tubes for respiratory failure in 18 months.

40/40

PROVIDERS TRAINED BEFORE
EACH GO-LIVE

79

PATIENTS SUPPORTED WITH BIPAP
SINCE JUNE 2024

0

INTUBATIONS FOR RESPIRATORY
FAILURE
SINCE APRIL 2025

6 of 6

UNITS CARRYING A
HAMILTON T1

WHY WE BUILT IT

The hospital is a long way away.

Greene County covers 658 square miles of the northern Catskills. Columbia Memorial Health in Hudson is our closest hospital, and for many patients it is the right destination. But the nearest Level I trauma center, cardiac catheterization lab and tertiary-level critical care are at Albany Medical Center, and from much of the county that is more than 80 minutes by ground. Our paramedics do not simply package a patient and go. Depending on where the call is, they may be managing that patient for an hour or more, in the back of a local ambulance, through mountain roads and winter weather.

For a patient who cannot breathe on their own, that hour matters. Before 2023, our tools were a bag-valve mask, a simple CPAP mask, and a basic pneumatic transport ventilator, the PalmVent, that could hold a rate and a volume and not much else. Bag-valve ventilation by hand, however skilled, cannot deliver the consistent volumes, pressures and oxygen targets that protect injured lungs and a recovering brain. A transport ventilator can.

So in 2023 we started with three, placed in the cars that could reach the most of the county through immediate response and system-status moves, and trained every provider to use them. The idea had to earn its way into the rest of the fleet.



Top: Medic 7 with Town of Coxsackie EMS and Albany Med LifeNet. Air transport is weather-dependent; the ventilator is not.

THE FLY CAR MODEL

Five cars. Six ventilators. One county.

Greene County Paramedics is an ALS first-response agency. We do not transport. Our paramedics respond in Chevy Tahoe fly cars from five stations, meet the local ambulance service on scene, and ride with the patient to the hospital, bringing advanced care that would otherwise not be available in the county.

Today every unit, including the Chief's vehicle, carries a Hamilton T1 transport ventilator. Whichever car is closest brings a full critical-care airway capability with it.



Left: the T1 with the bag-valve mask that is always within reach beside it. Right: the full setup, ventilator and monitor at the head of the stretcher, in the cramped quarters of a small ambulance on a real transport.



Built in stages, not bought in a day.

BEFORE 2023

A bag and a CPAP mask

Like most agencies, GCEMS managed breathing with a bag-valve mask for patients who had stopped breathing and a simple CPAP mask for patients who were struggling. CPAP helped, but it had one setting and no way to breathe for a patient who was wearing out. A basic pneumatic transport ventilator, the PalmVent, was carried for the rare patient who needed more, but it offered little control and no monitoring.

APRIL 2023

Columbia Memorial Health donates LTV 1200 ventilators

The donation made a true program possible. Every provider completed mandatory training over the following two months.



GCEMS Board President Mark Evans and Chief of Operations Steve Near accept the LTV 1200 ventilators from Columbia Memorial Health representatives.

JUNE 2023

LTV 1200s go into service

Three ventilators for six cars, placed in Medics 3, 5 and 9 to cover the most ground through immediate response and system-status moves. The first patient was ventilated on June 30, two days after the units went into the field.



Then and now: an LTV (this unit is the LTV 1000 used for training and never placed in service) beside the Hamilton T1 that replaced it.

2023

REMO Agency of the Year • ALS Provider of the Year

Regional Emergency Medical Organization honors for the agency and for Paramedic Shane Merchant.

MARCH 2024

First cardiac arrest patient survives to go home

Resuscitated in the field, ventilated on the LTV 1200 through transport, discharged from the hospital with no neurological deficit.



APRIL - JUNE 2024

Hamilton T1s arrive; three go live June 6

A year of results, including a full cardiac arrest reversal and a run of good outcomes, had proven the theory: prehospital ventilators were useful and good for patients. The agency upgraded. All 40 providers trained before go-live. T1s went into Medics 3, 5 and 9, with the LTVs moving to Medics 1, 7 and 8 so that every car now carried a ventilator. The T1 added BiPAP, a capability the LTV did not have. First T1 patient: two days after go-live.



The first three Hamilton T1s, and deployment day. Every ventilator carries its own set of GCEMS reference cards: ideal body weight and tidal volume, initial settings by scenario, and alarm response.



2024

REMO Excellence in Quality and Innovation • Harriet C. Webster Award for Leadership

The ventilator program recognized regionally; the Webster Award presented to Chief of Operations Steve Near.

JANUARY 1, 2025

Full fleet on Hamilton T1

Three more T1s purchased and placed in service the same day. The LTV 1200s retired. Six ventilators, one in every car.



Adaptive Support Ventilation on the T1, one of five modes in service across the fleet.

APRIL 2025

Last intubation for respiratory failure to date

Since then, every patient in respiratory failure who was a candidate for BiPAP has been managed without a breathing tube, more than 50 patients and counting.

2025 AND BEYOND

The region follows

Shortly after GCEMS's results became known, agencies across the region began purchasing transport ventilators. Most agencies in the REMO region are now implementing a ventilator program of some kind, including Windham and Catskill here in Greene County, both in 2025.

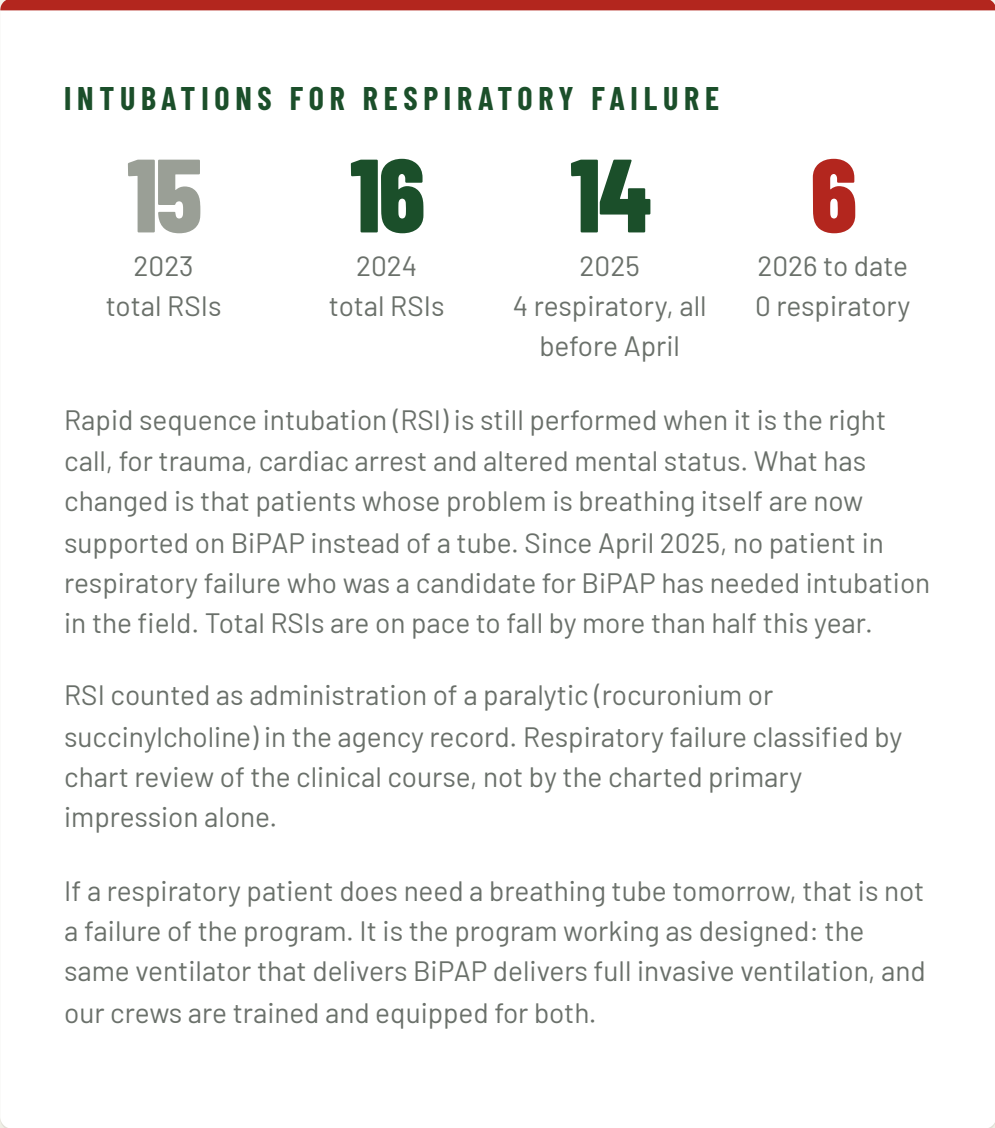
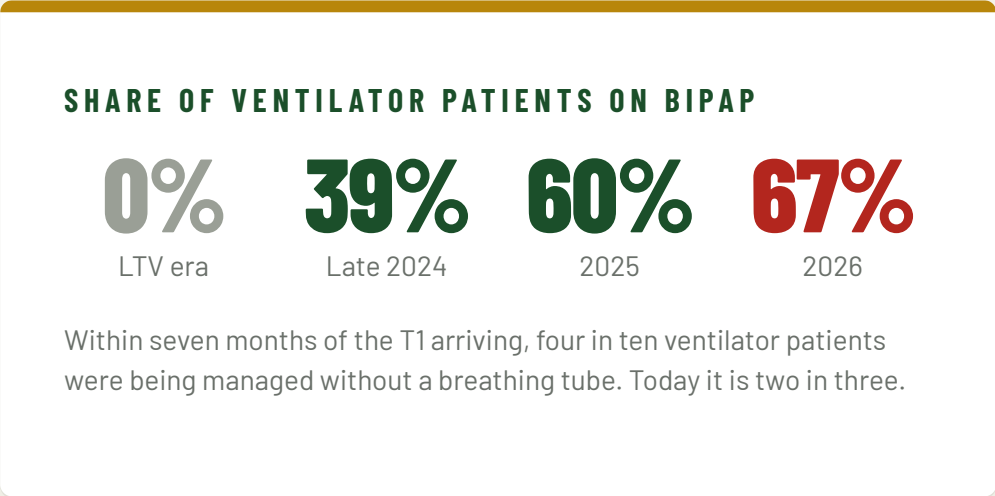
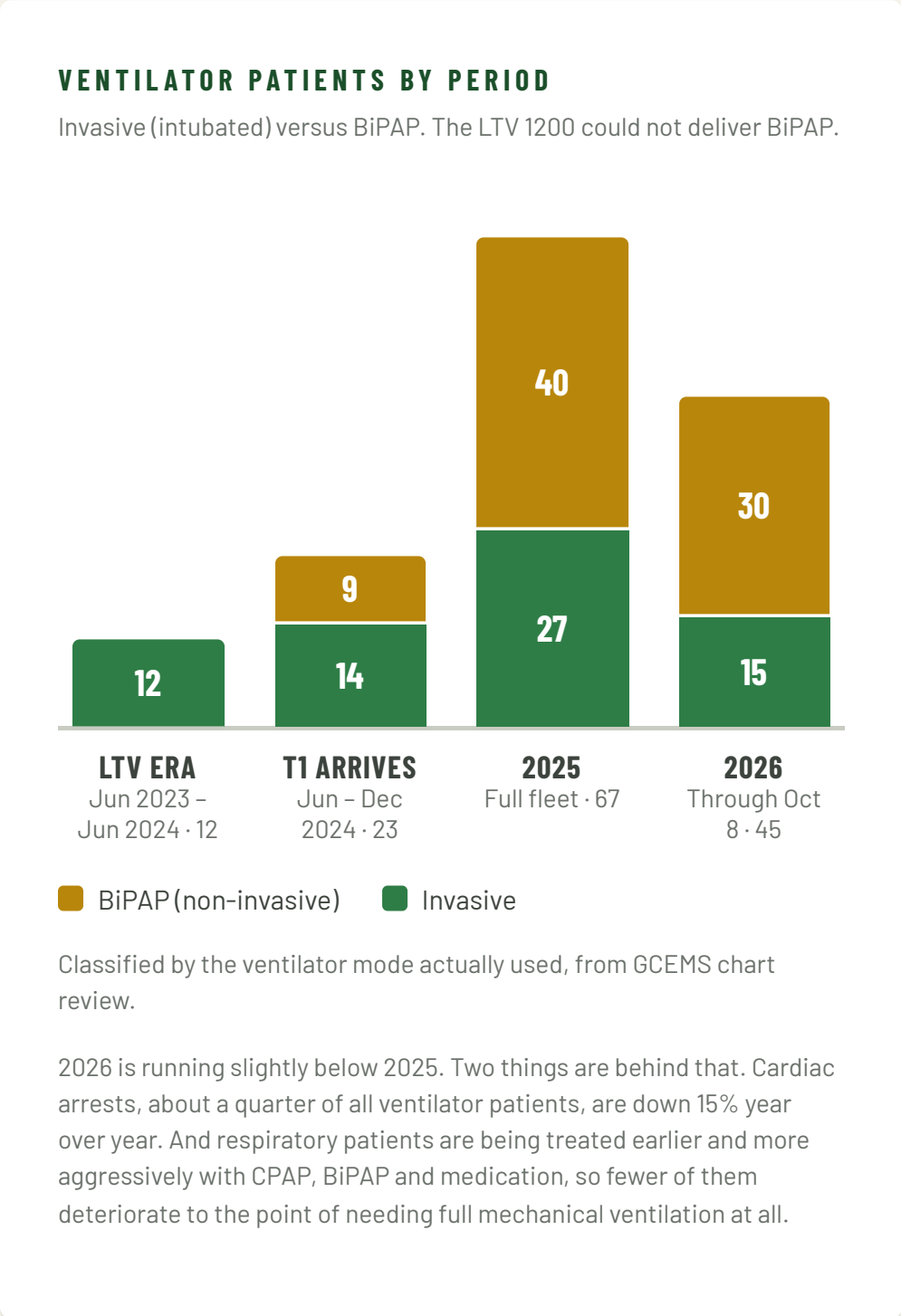


More patients helped. Fewer breathing tubes.

The Hamilton T1 did not just replace the LTV. It added a capability, non-invasive ventilation (BiPAP), that opened the ventilator to a whole group of patients who previously had two options: struggle to breathe on their own, or be sedated and intubated.

What avoiding a breathing tube means for a patient

A patient who is intubated in the field is sedated, often paralyzed, and arrives at the hospital on a ventilator they will later have to be weaned from, a process that can take days in an ICU and carries its own risks. A patient who arrives on BiPAP is awake, breathing with support rather than for them, and can often be managed less invasively from the moment they come through the door.



Fewer arrests. The same number going home.

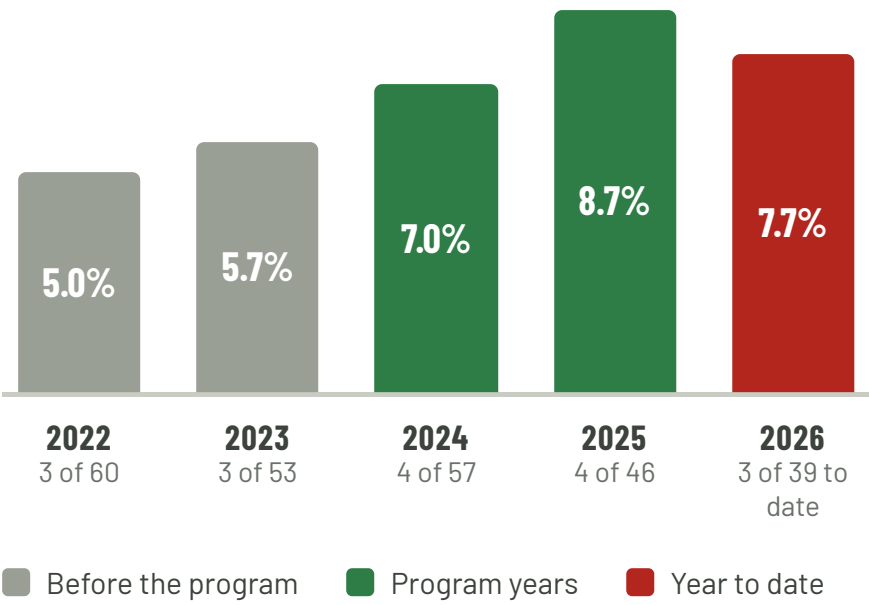
Cardiac arrest responses in Greene County have fallen by a third since 2022. The number of patients who survive to leave the hospital has not: three to four every year. As a share of arrests, survival to discharge has climbed from one in twenty to nearly one in eleven.

Part of that decline is the county's. Part of it, we believe, is ours. A patient in respiratory failure who is not supported in time can deteriorate into cardiac arrest on the way to the hospital. Those patients are now stabilized on BiPAP before they ever reach that point. A patient who never codes does not appear in any survival statistic, but they are the best outcome this program produces.

For the patients who do arrest, controlled ventilation after the heart restarts, with the right oxygen and CO₂ targets held steady for the whole ride, is one of the things the program was built to deliver. It is one piece of a chain that starts with bystander CPR and ends in the ICU. We are proud to be a stronger link in it.

SURVIVAL TO HOSPITAL DISCHARGE AFTER CARDIAC ARREST

Survivors ÷ cardiac arrest responses, by year.



These are small numbers. One patient moves the rate by about two points in either direction, so the trend across five years matters more than any single year's figure.

Four moments that showed us what the program could do.

MARCH 2024 · LTV 1200

Home without deficit.

A woman in her early fifties collapsed at home with no warning and no prior symptoms. She was in ventricular fibrillation when paramedics arrived. They resuscitated her on scene, placed her on the LTV 1200, and managed her as a critical care patient for the entire transport to Albany Medical Center, where she went to the cath lab. She went home with no deficit. It was the program's first survivor, and the first time we looked at each other and thought: we are onto something.

2024 · EARLY BIPAP

Breathing without a tube.

A patient with a history of severe allergic reactions and asthma, who had been intubated twice before for the same thing, was found barely conscious and struggling to breathe. Paramedics treated aggressively with BiPAP and medication, and the patient improved through the transport, arriving awake. An hour after handoff the BiPAP mask came off. The patient was discharged that same evening, never having been intubated. One of the first patients managed on the T1's non-invasive mode, and the clearest example of what it changed.

2026 · INVASIVE VENTILATION

When the tube is the right call.

A cyclist went into ventricular fibrillation on the side of the road. Paramedics defibrillated him, intubated him, and placed him on the ventilator. Albany Med LifeNet landed, moved him to the aircraft's ventilator using our settings, and flew him out; the handoff was seamless. He has made a full recovery. The same program that keeps breathing tubes out of respiratory patients is the one that makes an intubated transport, and a transfer between crews, safe when there is no other option.

APRIL 2025 → TODAY

Eighteen months, zero.

Since April 2025, not one patient in respiratory failure has needed a breathing tube in the field. More than 50 patients have been managed on BiPAP in that time. Every one of them avoided sedation, paralysis, and the days of ventilator weaning that follow an intubation.



Left: ALS Coordinator Mark Foster, FP-C, demonstrates BiPAP with Paramedic William O'Connor as the volunteer. Right: the full setup during a real BiPAP transport.

Where the ventilator has gone to work.

From the Hudson River towns to the mountaintop, this is not a capability that lives at one station. Cairo and Cocksackie lead the count, but nearly one in four patients was on the mountaintop, in Hunter, Prattsville, Lexington, Jewett, Ashland or Windham, where the hospital is furthest away and the ventilator matters most.

145

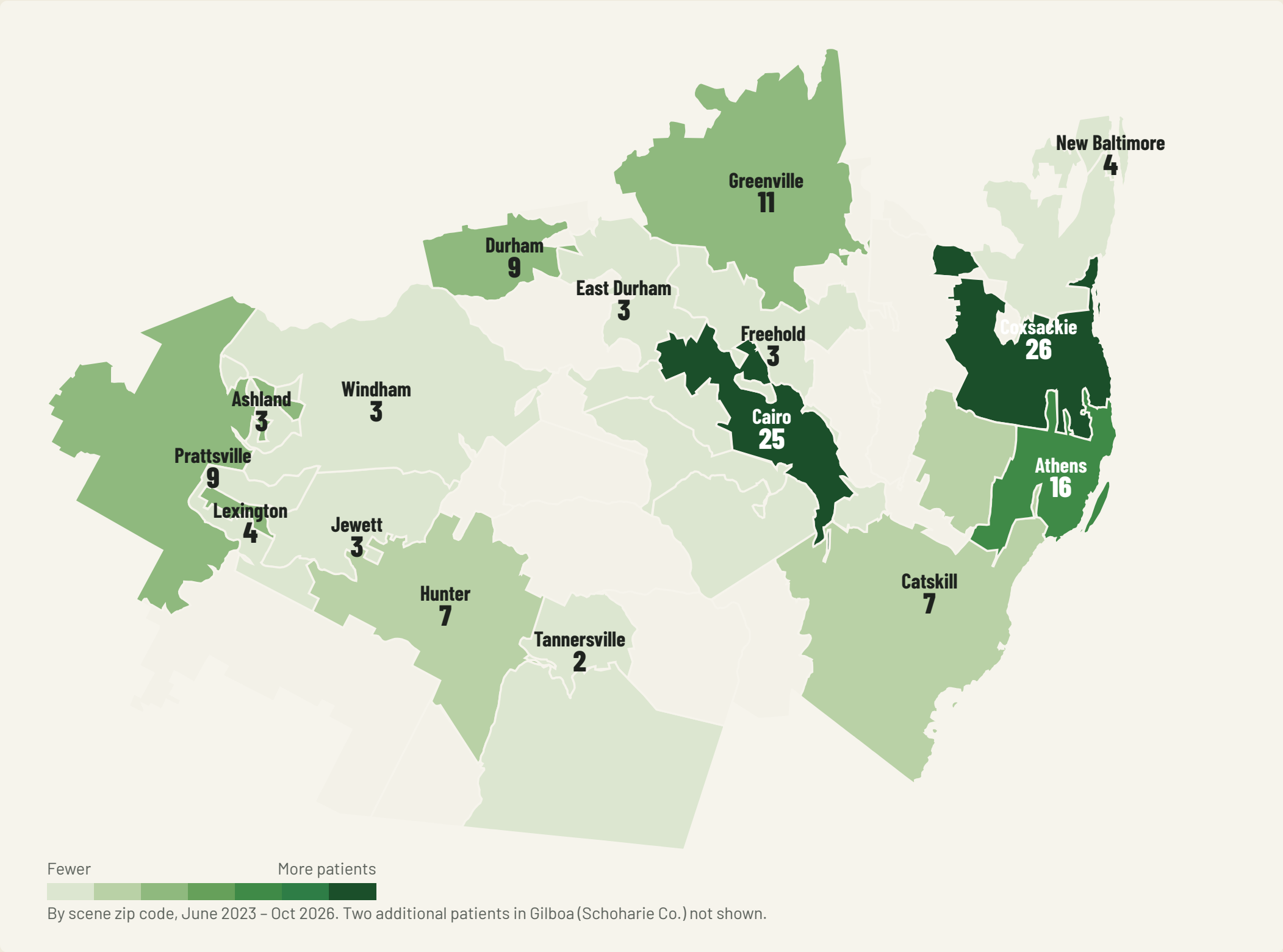
VENTILATOR PATIENTS WITH A
RECORDED SCENE LOCATION

13 of 14

GREENE COUNTY TOWNS WHERE
A PATIENT HAS BEEN VENTILATED

23%

OF PATIENTS ON THE MOUNTAINTOP,
FURTHEST FROM THE HOSPITAL



Counts by scene zip code from agency records, June 2023 through October 2026. Hover or tap a zip code for its count. Halcott, the one town not shown with a patient, has a population under 300.

TRAINING

Forty of forty. Twice.

Both rollouts followed the same rule: nobody touches a patient with a ventilator they have not been trained on. In 2023 and again in 2024, every one of the agency's 40 providers completed mandatory training before the equipment went into service, with instruction from Hamilton Medical and LifeNet of New York, hands-on practice and simulation.

The training did not stop at go-live. GCEMS now runs two ventilator workshops a month: small groups of four working through real scenarios and troubleshooting on the T1, with a required number of sessions per provider each year. The skill is kept, not just earned.

Every ventilator carries a set of GCEMS-written reference cards: ideal body weight and tidal volume tables, initial settings by clinical scenario, alarm response, and the one rule above all others: if the ventilator cannot be made to work, disconnect and ventilate by hand.



Left: Jimmy McMullen, Hamilton Medical, leading T1 training at GCEMS. Right: hands-on practice, and the T1 at a community outreach event.

40/40

PROVIDERS TRAINED BEFORE EACH GO-LIVE

2 / month

ONGOING SMALL-GROUP VENTILATOR WORKSHOPS, WITH REQUIRED ATTENDANCE

THE PEOPLE BEHIND IT

A program this size does not happen alone.



Role reversal: Chief of Operations Steve Near walking former Chief Stephen Brucato through the T1. The training was mandatory for everyone, and nobody was grandfathered in.

MEDICAL DIRECTION

Dr. Craig A. Stanger

Medical Director, GCEMS. Set the clinical parameters, the agency-level pressure ceilings and the protocol framework the program runs on.

CLINICAL LEADERSHIP

Mark Foster, FP-C

ALS Coordinator, GCEMS. Training, credentialing and day-to-day clinical oversight of the ventilator program.

OPERATIONS

Patricia McAneny

Deputy Chief of Operations, GCEMS, and one of the agency's most tenured paramedics.

GOVERNANCE

GCEMS Board of Directors

Led by Board President Mark R. Evans. The board backed the program from the first donation through the purchase of six Hamilton T1s.

TRAINING PARTNER

LifeNet of New York

Albany Med's critical care flight program. Their crews were a major part of the initial ventilator training, and they are the ones we hand our patients to when they fly.

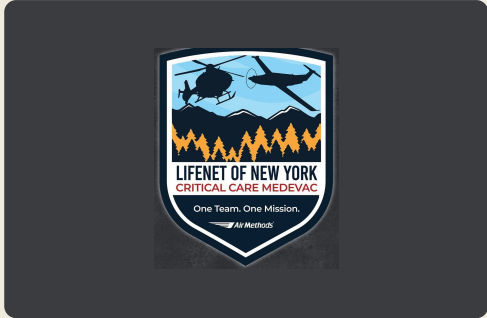
OUR CREWS

40 paramedics and providers

Who trained on their own time, learned a new way to manage breathing, and have now used it 147 times for the people of Greene County.

OUR PARTNERS

Built with the people who showed up.



The ventilator was the first step toward critical care in the field.

The ventilator program proved that a rural fly-car agency could carry hospital-level capability and use it safely. It set the pattern the programs that follow it are built on: equipment, mandatory training, written protocols, medical direction.

Rapid sequence intubation

A formal RSI program guide, now aligned to NYS protocols v26.2, so that when a tube is the right call, it is done the same way every time.

Infusion pumps

Programmable infusion pumps for precise medication delivery over long transports, in preparation for service.

FROM THE CHIEF

Why I wanted to write this down.

When Columbia Memorial handed us those LTVs in the spring of 2023, I did not know if this would work. I knew the math: a county this size, a hospital this far, and a bag in someone's hand for an hour was not good enough. What I did not know was whether a fly-car agency of forty people could take on something that had mostly lived in flight programs and critical care transport, and do it well.

The March 2024 patient settled that for me. She went home. We were onto something.

Everything since then has been the crews. They did the training on their own time, twice. They learned a new way to manage breathing, got good at it fast, and have now used it 147 times for their neighbors. I am proud of them, and I wanted the county to know what they have built.

Steve Near

CHIEF OF OPERATIONS, GREENE COUNTY PARAMEDICS



GREENE COUNTY PARAMEDICS

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Figures reflect agency records June 28, 2023 through October 8, 2026. Patient stories are shared without identifying details. Hamilton T1 and Hamilton Medical are trademarks of Hamilton Medical AG.